

geha-coverage-policy-cyramza.pdf

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Source PDF page 1

Cyramza (Ramucirumab)

Original Effective Date: 11/2022

Revision Date(s): 11/2022,12/2022, 7/2023,4/2024, 5/27/2025,11/17/2025

Review Date: 11/17/2025

Policy type: Medical Necessity

Line of Business: All GEHA Medical Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability.

Indication Specific Criteria:

Hepatocellular Carcinoma

The member has unresectable, locally advanced, or metastatic disease; AND

Cyramza is used as subsequent therapy; AND

Cyramza is used as a single agent; AND

The member's liver functional status is Child-Pugh Class A1; AND.

The member's AFP (alpha-fetoprotein) is 400 ng/mL

Non-Small Cell Lung Cancer

The member has recurrent, advanced, or metastatic disease; AND.

o Cyramza is used in combination with docetaxel; AND

Used as subsequent therapy; AND

Member has an ECOG of 0-22; OR

o Cyramza is used in combination with erlotinib; AND

Member has EGFR exon 19 deletion or exon 21 (L858R) mutation; AND

Used as first-line therapy; OR

Used as continuation of therapy following disease progression on erlotinib and ramucirumab IF:

Member has asymptomatic disease; OR

Member has symptomatic brain lesions; OR

Member has symptomatic systemic limited metastases and T790M mutation negative; OR

Source PDF page 2

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

National Comprehensive Cancer Network (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; OR

United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with the product labeling; OR

Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:

O Clinical Pharmacology (Strong For); OR

O Wolters Kluwer Lexi-Drugs (Level A); OR

Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:

O American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the European Journal of Cancer and Clinical Oncology); Gynecologic Oncology; International Journal of Radiation, Oncology, Biology, and Physics; The Journal of the American Medical Association; Journal of Clinical Oncology; Journal of the National Cancer Institute; Journal of the National Comprehensive Cancer Network (NCCN); Journal of Urology; Lancet; Lancet Oncology; Leukemia; The New England Journal of Medicine; Radiation Oncology;

O Meeting abstracts and case reports are excluded from consideration;

Non-standard protocols may be approved based on unique clinical circumstances;

Dose and frequency should be consistent with United States Food and Drug Administration (FDA) labeling, National Comprehensive Cancer Network (NCCN), or indication specific peer-reviewed literature;

In the instance that a request is made for drug(s) that was (were) previously tried (including in the same pharmacologic class or with the same mechanism of action) and such drug(s) was (were) discontinued due to a lack of efficacy the request may be subject to an off-label review for medical necessity unless supported by the NCCN or high quality literature (prospective phase 2 or 3 studies published as full manuscripts in a CMS-supported journal).

Billing

Drug Name	HCPCS Code	Description
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Cyramza	J9308	Injection, ramucirumab, 5 mg

Source PDF page 3

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References

1. Cyramza [package insert]. Eli Lilly and Company, Indianapolis, IN. Available at: https://www.accessdata.fda.gov/drugsatfda_docs/label/2025/125477s049lbl.pdf 2. Cyramza. NccN Drugs & Biologics Compendium. Available at:

https://www.nccn.org/professionals/drug_compendium/content/

3. Referenced with permission from the NccN Clinical Practice Guidelines in Oncology (NCCN Guidelines). National Comprehensive Cancer Network, Inc. 2025. All rights reserved. Accessed November 12, 2025. To view the most recent and complete version of the guideline, go online to NCCN.org.

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Date	Updates
11/7/22	Policy approved by P&T
12/12/22	Updated formatting and incorporated universal criteria
7/19/23	Updated Hepatocellular Indication Specific Criteria (from Hepatobiliary). Added Approval Criteria. Added Mesothelioma: Pleural NccN Guidelines to References.
4/5/24	Annual Review. Updated Universal Criteria. Updated references.
5/27/2025	Annual Review. Update policy template including universal approval criteria, disclaimer; approved at P&T 5/27/2025
6/5/2025	Policy adopted by GEHA; Document assessed for universal accessibility
11/17/2025	Removed Ramucirumab/Pembrolizumab - approved at OH P&T